

SOUTH SUDAN HUMANITARIAN UPDATE



The Humanitarian Coordinator in South Sudan, Ramanathan Balakrishnan, engages with a health worker during a mission to Akobo Town on 4 June 2026.

Photo: DSRSG/RC/HC Office/ Yuta Funakoshi

As of 7 June 2026

HIGHLIGHTS

- The response in Akobo has significantly scaled up, with over 430 MT of food delivered, reaching about 60,000 of the 80,000 targeted people (75 per cent).
- The Humanitarian Coordinator in South Sudan, together with the UNHCR Regional Director, visited Akobo Town on 4 June and engaged with the local authorities, humanitarian partners, communities, and UNMISS on the security and humanitarian situation.
- Humanitarian access across South Sudan remains constrained. On 25 May, five contractors were killed during fighting between armed groups in Morobo County, Central Equatoria State, while working on a bridge construction project supported by a UN agency.
- Although no Ebola cases have been confirmed in South Sudan, the risk of an outbreak remains high due to porous borders and weak health systems.
- Flooding has already begun affecting vulnerable communities in Unity State, damaging shelters, disrupting livelihoods, and increasing humanitarian needs.

Humanitarian Situation and Scale-Up in Akobo County

Fragile and volatile situation

The humanitarian situation in Jonglei State, particularly in Akobo County, remains fragile and volatile. While relative calm has prevailed in recent days, localized violence, persistent protection concerns and continued population movements are driving humanitarian needs and placing additional pressure on already overstretched services. The health situation remains particularly concerning. Akobo County Hospital is currently serving an estimated 267,000 people, more than five times its intended capacity, and faces severe constraints in infrastructure, water supply and medical equipment. At the same time, Akobo West has virtually no functional health services, leaving large segments of the population without access to basic healthcare. Schools face critical teacher shortages, and WASH conditions are deteriorating due to the lack of solar power for the area's only water pumping station.

Findings from recent assessments in Chuil in Nyirol County, Yuai in Uror County, and Paguir in Fangak County in Jonglei State highlight widespread distress and significant coping challenges. Early and forced marriage has emerged as a key concern, alongside notable gaps in mental health and psychosocial support (MHPSS) and child protection services. In high-risk areas, communities continue to rely on informal coping mechanisms due to limited humanitarian coverage. The collapse of local markets, driven by conflict and looting, has severely constrained access to food supplies.

High-level mission to Akobo Town

On 4 June, the Humanitarian Coordinator in South Sudan, Ramanathan Balakrishnan, together with the UNHCR Regional Director, Mamadou Dian Baldé, conducted a mission to Akobo Town to engage local authorities, humanitarian partners, communities and UNMISS on the security and humanitarian situation. Discussions focused on access constraints, protection risks and ongoing response efforts for conflict-affected populations. The delegation commended local authorities for supporting humanitarian operations in Akobo and surrounding areas, while underscoring the urgent need to ensure safe passage and protection of humanitarian convoys, particularly along contested road routes and the Sobat River, to facilitate timely assistance to Akobo East and West. The delegation also called for guarantees to prevent further destruction and looting of humanitarian assets, reportedly carried out mainly by local youth. Sustained security assurances will be critical to maintaining operations, particularly during the rainy season.

Over 420,000 people displaced

Between January and May, more than 420,800 people were displaced across South Sudan due to conflict, according to IOM Displacement Tracking Matrix figures. This includes over 324,100 people displaced by violence in Jonglei State: 261,570 within Jonglei, nearly 29,000 to Mingkaman in Awerial County, Lakes State, over 27,000 to Nasir, Panyikang and Ulang counties in Upper Nile State,

and more than 6,000 to Juba County in Central Equatoria State.

In addition to displacement, a significant returnee population was reported in Akobo Town and surrounding counties in May, many arriving from Tiergol, Ethiopia. Partners are verifying these figures. These return movements are largely driven by the dire living conditions in refugee-hosting areas. According to IOM/Displacement Tracking Matrix, the return process is fraught with challenges, as many individuals are relocating to regions where the South Sudan People's Defence Forces (SSPDF) have recently established control, in search of safety and basic services rather than permanent resettlement. Sustained United Nations Mission in South Sudan (UNMISS) presence and patrols are critical to deter violence, protect civilians and humanitarians, and maintain early warning systems in high-risk areas. On 3 June, the Relief and Rehabilitation Commission (RRC) in Jonglei State urged humanitarian partners to resume operations in Urur, Nyirol and Akobo West Counties. Local authorities committed to ensuring the safety and security of humanitarian personnel, assets and supplies as partners resume operations in areas previously affected by violence.

Akobo hospital operates at five times its capacity

Akobo County Hospital continues to face extreme operational pressure. The facility is currently serving an estimated 267,000 people, nearly five times its intended capacity of approximately 50,000 people. Critical gaps persist, notably the availability of only one functional delivery bed, unreliable water supply, limited infrastructure, and a non-functional generator. Akobo hospital urgently requires additional medicines, Inter-agency Emergency Health Kits (IEHKs), oxygen support, power restoration support, cholera preparedness supplies, and surge staffing to sustain the rapidly increasing caseload. IEHK supplies remain prepositioned in Bor awaiting delivery. UNICEF is providing support to restore the hospital's power supply. Akobo West remains largely without functional health services. WHO is working with three partners including Medair, CIDO, and MSF to support emergency reopening of primary health-care centres.

Scale up of emergency response in Akobo East

As of 4 June, Akobo County is calm, and humanitarian partners have rapidly expanded their operations. Nearly 30 humanitarian organizations are active in the area. Over the past three weeks, more than 60,000 of the targeted 80,000 people in Akobo East—equivalent to 75 per cent—have received food assistance, including over 15,000 people through emergency food distributions. Nutrition interventions have supported more than 6,000 women, while over 30,000 vulnerable people have received high-energy biscuits. Verification and general food distributions targeting conflict-affected populations, including internally displaced people (IDPs) and returnees, are ongoing.

This scale-up has been made possible through a substantial logistical effort. More than 200 metric tons of supplies have been delivered through UNMISS-supported convoy operations, while over 60 UNHAS flights have transported approximately 430 metric tons of humanitarian cargo and more than 200 personnel into the area. UNHAS flight frequency has increased to three rotations per week. Additional support has included the delivery of 4 metric

tonnes of medical supplies to Akobo hospital on 27 May and 1.2 metric tonnes of sexual and reproductive health kits in the past week.

Partners are increasingly expanding interventions across sectors, including agriculture, fishing, health, nutrition, water, sanitation and hygiene (WASH), education, protection and non-food assistance. Plans are underway to extend integrated health and nutrition services to peripheral facilities in neighbouring payams. Schools in Akobo Town have reopened and learner registration is ongoing; however, the continued absence of teachers remains a major constraint to education provision.

Priority humanitarian actions include sustained support for returnees, faster delivery of supplies and increased UN Humanitarian Air Service flights to Akobo. However, partners are concerned that the delivery window is closing rapidly as the rains have already begun and will soon intensify, making road cargo movement impossible.

Convoy delivering over 300 metric tonnes of supplies arrives in Akobo Town

On 31 May, a 33-truck convoy organized by WFP and the Logistics Cluster delivered more than 210 metric tonnes of food and nutrition assistance, alongside 100 metric tonnes of relief items. The Logistics Cluster in Bor has also begun loading additional relief supplies for onward river transport to Akobo via Malakal along the Sobat corridor, as road access remains largely impassable due to flooding and poor conditions. Key routes, including the Pibor–Likuangole–Akobo eastern corridor and the Pathai–Ayod route in northern Jonglei, are currently inaccessible.



People affected by conflict receives food assistance in Akobo Town, Jonglei State.

Photo: OCHA/Oleksandr Osinsky

Protection concerns continue to worsen

Protection monitoring reports indicate increasing incidents of child abduction, conflict-related sexual violence, rape and recruitment into armed groups perpetrated by parties to the conflict. As of April, UNICEF had identified more than 500 unaccompanied children across Jonglei State, although the actual number is likely to be significantly higher.

"As we were fleeing from the fighting and aerial attacks, we were ambushed by gangs, and I got separated from my two youngest daughters. They are just 9 and 6 years. I didn't see where they went and I didn't see them harmed. I have had no news since, and I don't know where they are. I am desperate to find my girls again", Yang Chol, mother of 4 children.



The Humanitarian Coordinator, Ramanathan Balakrishnan and the UNHCR Regional Director Mamadou Dian Baldé engages with aid workers and local communities in Akobo Town.

Photo: DSRSG/RC/HC Office/Yuta Funakoshi

Humanitarian access impediments constraints aid operations

Humanitarian access across South Sudan remained constrained during the reporting period, particularly in Northern Jonglei, Upper Nile, Unity, Central Equatoria and Eastern Equatoria. Between 21 and 28 May, a total of 19 access-related incidents were documented, compared with 10 the previous week, reflecting a significant increase in humanitarian access constraints and operational risks. Cumulatively, 327 humanitarian access incidents recorded between January and May 2026, representing nearly 74 per cent increase compared to the same period in 2025.

Increased violence against humanitarian personnel

On 25 May, reports indicate that five contractors including one foreign national and four South Sudanese nationals were allegedly killed during fighting between armed groups in Kendila in Morobo County, Central Equatoria State while working on a bridge construction project reportedly supported by a UN agency. At least 19 fatalities recorded since January, alongside 183 incidents affecting personnel and humanitarian assets.

On 26 May, two humanitarian staff members conducting a humanitarian distribution mission were reportedly abducted by an armed group in Yei County, Central Equatoria State. The incident occurred after the vehicle transporting the team was stopped by the armed actors, who subsequently took the two staff members to an unknown location. The abducted staff members were released on 27 May. However, both individuals were reportedly subjected to severe physical torture during captivity. In a separate incident in Yei County on 1 June, two humanitarian staff members were abducted while undertaking a field mission and safely released after a day. Between 29 May and 1 June 2026, two incidents involving the attack and robbery of humanitarian staff were also reported in Mundri West County, Western Equatoria State. During the same period, UN personnel in Kapoeta South County, Eastern Equatoria State were reportedly stopped at gunpoint and robbed of their personal belongings. These incidents highlight the growing security risks faced by humanitarian actors and are likely to constrain humanitarian access, disrupt field operations, and increase operational costs.

Humanitarian access restrictions

Pressure on humanitarian organizations operating in opposition-controlled areas to relocate services to government-controlled areas persisted. Authorities in Nasir County reportedly instructed humanitarian organizations to relocate Area Managers to Nasir town, with a strict deadline of 29 May to confirm compliance with headquarters.

Government-appointed local authorities of Longochuk County, operating from Mathiang, reportedly denied a humanitarian assessment mission access to Udier, where approximately 22,590 conflict-displaced people are sheltering and are reported to be in urgent need of humanitarian assistance. The denial of access prevented humanitarian actors from conducting needs verification and assessment activities, likely delaying the delivery of critical life-saving assistance and services. The restriction also affected the distribution of food assistance that had already been prepositioned in Mathiang for onward delivery to affected populations in Udier, potentially exacerbating humanitarian vulnerabilities among displaced households.

Additional incidents included labour authorities in Maban County reportedly directing humanitarian organizations to deduct 10 percent from casual labourers' daily wages, looting of a humanitarian health facility in Doro Refugee Camp, theft of nutrition supplies from an NGO warehouse in Renk, and threats by SSPDF personnel against humanitarian staff in the Malakal former PoC site linked to child protection activities supported by UN agencies.

In Jonglei State, on 20 May, the SPLA-iO Commissioner in Akobo East warned humanitarian actors to vacate Akobo West, potentially affecting over 94,000 conflict-affected people and halting assistance. In Unity State, increased checkpoints and ambushes along key routes, including an attack on a UN agency convoy, have heightened risks and further constrain humanitarian access. In Eastern Equatoria, interference with humanitarian movements has caused operational delays and raised safety concerns.

Insecurity and deteriorating road conditions disrupt aid operations

In Jonglei State, continued insecurity and deteriorating road conditions significantly disrupt humanitarian operations. Ongoing clashes between opposition forces and the SSPDF in Nyirol and Uror counties disrupted and delayed the delivery of critical humanitarian assistance to hundreds of thousands of conflict-affected and displaced populations. Violence targeting humanitarian personnel and assets also persisted, further undermining humanitarian operations. In addition, the onset of the rainy season has rendered key supply routes increasingly impassable, creating major logistical challenges for the movement of humanitarian personnel and relief commodities. These constraints are impeding efforts to deliver life-saving assistance to vulnerable populations in Northern Jonglei counties and the Greater Pibor Administrative Area, where humanitarian needs remain acute.

Health risks: disease outbreaks and Ebola preparedness

High risk of Ebola importation

South Sudan remains at high risk of Ebola Virus Disease (EVD) importation following the evolving Bundibugyo Virus Disease outbreak in the Democratic Republic of the Congo (DRC) and Uganda. Preparedness and readiness activities continue under the leadership of the Ministry of Health with support from WHO, Health Cluster partners, and humanitarian actors.

Reassessment of national readiness against the WHO checklist was conducted on 4 June. Preliminary results indicate the improvement in readiness to 45 per cent from the previous assessment (29 per cent against the WHO checklist) conducted on 18 May; however, further efforts are needed, including Infection Prevention and Control (IPC), contact tracing, and surveillance. Also, strengthening the sub-national level readiness and preparedness remains critical. Fifteen priority counties have been identified, including Yei, Morobo, Kajo-Keiji, Tambura, Torit, and Juba. All the EVD response pillars are now activated. The Minister of Health and a high-level technical delegation from South Sudan participated in the High-Level Cross-Border Meeting in Uganda on 22 and 23 May.

The Ministry of Health has activated daily coordination meetings to ensure continuous monitoring, planning and operational readiness. WHO and partners continue to support the Ministry of Health to strengthen surveillance, IPC, points of entry preparedness, case management readiness, and community engagement, particularly in high-risk border and movement corridors. Despite these efforts, significant gaps remain, particularly in water, sanitation, and hygiene services, which are critical for both cholera control and Ebola preparedness.

Government allocates vehicles to support response efforts

On 2 June, the Senior Presidential Envoy for Special Programs, Adut Salva Kiir, has donated 10 vehicles to support national Ebola preparedness and response efforts in the country. In addition, on 26 May, the Ministry of Health announced a 90-day budget of US\$7.3 million to boost preparedness and response measures against a potential Ebola outbreak.

Worsening cholera outbreak

The cholera outbreak continues to worsen. Cholera remains a major concern, with more than 104,813 cases reported nationwide since 2024. In the last 14 days of reporting (onset from 19 May to 1 June), 660 new cases and three deaths (CFR: 0.5%) were reported in five counties. Most of these cases came from Rubkona (465, 70.5%), Mayom (131, 19.8%), Renk (40, 6.1%) and Panyijiar (16, 2.4%). The outbreak is increasingly concentrated in Unity, Lakes and Upper Nile states, largely driven by conflict and poor water, sanitation and hygiene (WASH) conditions, particularly among displaced, returnee and refugee populations. The conditions in Bentiu IDP camp remain particularly worrying given population density and persistent WASH challenges. In the past seven days

(26 May-1 June), a total of 221 new cholera cases and two deaths (Case Fatality Rate: 0.9%) were reported across five counties, representing a 38.1 per cent decrease compared to the previous week (358 cases). Unity State remains the most affected, accounting for 34 per cent of all cholera cases, and a recent outbreak in the Bentiu IDP camp, home to over 100,000 people. On 31 May, 62 new admissions were recorded in Bentiu IDP camps, with 25 cases at Bentiu State Hospital CTC and 14 at Yaoanyang CTC—an increase from 29 May (24, 14, and 9 cases, respectively). Additionally, on 31 May, a severely ill and dehydrated patient was referred from Mayom County to Bentiu. The patient died upon arrival at the Bentiu IDP camp Cholera Treatment Centre. IOM is scaling up cholera response in Bentiu IDP Camp. Also, MSF complements IOM's response by providing desludging services, solid waste management, and a community clean-up campaign.

Flood preparedness and early impact

Flooding has already begun affecting vulnerable communities in the country. In Koch County, Unity State, rising river water levels flooded the area, impacting between 1,800 and 2,000 people, destroying over 300 shelters as well as food stocks and basic infrastructure. Response capacity remains limited.

While preparedness activities are underway, operational capacity remains limited as the country enters the peak rainy season. Flooding is expected to further complicate humanitarian access, increase disease risks and place additional strain on already vulnerable populations and response systems.

Flood preparedness efforts are ongoing through coordination between OCHA and the Ministry of Humanitarian Affairs and Disaster Management. The national Flood Taskforce, co-chaired by the Relief and Rehabilitation Commission (RRC) and OCHA has been activated. It is supported by the National Coordination and Operations Centre (NCOC). The 2026 flood Mitigation and Response plan is being developed.

Funding Update

Despite generous contributions from donors, funding levels remain insufficient to respond to simultaneous shocks across conflict, displacement, food insecurity, flooding, and disease outbreaks. The UN Emergency Relief Coordinator, Tom Fletcher, announced an allocation of \$12 million from the Central Emergency Response Fund (CERF) to support people affected by conflict-induced displacement and food insecurity in South Sudan on 13 May.

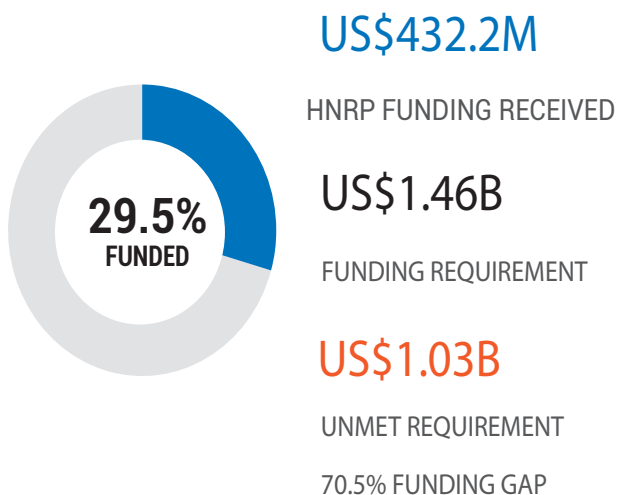
The funding will enable the United Nations and its humanitarian partners to deliver life-saving multi-sector assistance, to approximately 200,000 people. Assistance will target Akobo County in Jonglei State, as well as areas in Upper Nile State and other priority locations.

The South Sudan Humanitarian Fund (SSHF) is also preparing a complementary allocation addressing food insecurity. Separate US\$12.99 million CERF allocation approved for UNHAS to sustain humanitarian air operations through October 2026.

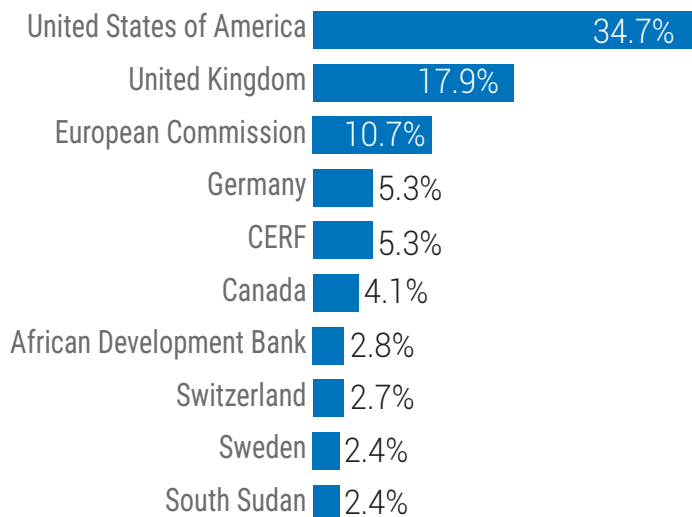
Against an estimated 10 million people in need, roughly two-thirds of the population, humanitarians reached approximately 2.3 million between January and April, 53 per cent of the 4.3 million targeted under the 2026 Humanitarian Needs and Response Plan

(HNRP). This was delivered with US\$382 million received, just 26 per cent of the US\$1.46 billion required. Funding shortfalls are limiting life-saving assistance across all sectors. Without urgent, flexible support, millions risk being left without critical aid.

FUNDING



TOP 10 DONORS



WFP delivered more than 430 metric tonnes of food items, reaching about 60,000 people in Akobo County in the past three weeks.

Photo: OCHA/Oleksandr Osinsky